

Pediatric Developmental Screening Report

M-CHAT Autism Spectrum Screening · Confidential Clinical Document

Patient Information

Patient Name	N/A (Self-Report)	Patient ID	ASD-2026-002
Date of Report	April 29, 2026	Exam Type	Developmental Screening (M-CHAT)
Age / Gender	7 years · Male	Ethnicity	White-European
Country of Residence	United States	Informant	Self
Jaundice at Birth	No	Prior Screening App	No

M-CHAT Screening Responses

Item	Question	Response
A1	Does the child look at you when you call their name?	Yes
A2	Does the child use pointing to show interest?	Yes
A3	Does the child engage in pretend play?	Yes
A4	Does the child show interest in other children?	Yes
A5	Does the child bring objects to show you?	Yes
A6	Does the child make eye contact with you?	Yes
A7	Does the child imitate your actions?	Yes
A8	Does the child respond to their name being called?	Yes
A9	Does the child use simple gestures?	Yes
A10	Does the child smile in response to your smile?	Yes

10 out of 10 items screened positive — maximum score attained.

Clinical Observations

This screening was completed by the patient himself, a 7-year-old male of White-European ethnicity residing in the United States. No history of neonatal jaundice is reported. The patient has not previously used a digital screening application. All ten M-CHAT items returned a positive response, representing the maximum attainable screening score. This pattern of responses is clinically significant and warrants immediate specialist attention.

Behavioral Findings

Social interaction: Positive screening across all social engagement items, including eye contact, response to name, and interest in peers.

Communication: All communicative items screened positive, including pointing, gesturing, and spontaneous language use.

Pretend and imitative play: Both pretend play and imitation of actions screened positive.

Overall profile: The uniformity of positive responses across all domains is atypical and strongly suggestive of a pervasive developmental condition.

Clinical Impression

The patient presents with a maximal positive M-CHAT screening profile (10/10 items). The breadth and consistency of flagged items across social, communicative, and behavioral domains is highly indicative of Autism Spectrum Disorder (ASD). The self-reported nature of this screening, while noted, does not diminish its clinical significance at this score level. Formal diagnostic evaluation by a licensed neurodevelopmental specialist is strongly recommended without delay. This screening result alone is not sufficient for a clinical diagnosis.

Recommendations

1. Urgent referral to a pediatric neurodevelopmental specialist for comprehensive ASD diagnostic evaluation.
2. Initiate early intervention and behavioral support services pending diagnostic confirmation.
3. Conduct a speech and language therapy assessment to evaluate communicative functioning.
4. Consider psychoeducational evaluation to assess cognitive and adaptive functioning.
5. Provide family or caregiver psychoeducation regarding ASD and available support pathways.

Examining Clinician

Date: April 29, 2026